THE DIGITALKART EDIT / INDIA EDITION

# The Indian Perimenopause & Menopause Playbook

2026-27

What your forties are actually doing to your body, what India's 2026 guidelines say about treating it, and the twelve-week plan that protects your bones, your heart and your sleep.

**46.2 yrs**
Average age at natural menopause in India

**87%**
Report joint and muscle pain - the top Indian symptom

**3x**
Bone loss accelerates around the final period

Inside: the full symptom map | hormone therapy, explained honestly | the 2026 Indian guidelines | calcium and protein in everyday Indian food | a 7-day menu with counts | a 12-week home strength plan | screening calendar 40-65 | trackers and doctor-visit sheets

# About this edition

This is a working handbook, not a textbook. It was written for Indian women in their forties and fifties who have been told that their aches, exhaustion and sleeplessness are 'just age', and who have not been given a plan. It brings together the Indian Menopause Society's 2026 clinical practice guidelines, the World Health Organization's current position, India's national nutrition and disease-screening guidance, and the Indian studies that show what this transition actually looks like here - which is not quite what the imported leaflets describe.

Every figure in these pages is traceable to the sources listed in Part 20. Where Indian and international guidance disagree - calcium intake is the clearest example - both positions are stated, with the practical reading that follows from them.

## How to read it

Parts 2 to 4 explain what is happening and how to get it checked properly. Part 5 is the hormone therapy chapter, written to be read with a doctor rather than instead of one. Part 6 covers every non-hormonal route. Parts 7 to 12 take one system at a time - bone, heart, sleep, mood, the genitourinary system, the body. Parts 13 and 14 are where you do the work: the kitchen and the strength plan. Part 15 handles work, family and money. Part 16 is your screening calendar. Part 17 is the paperwork. Part 18 is the list of things that need a doctor today.

## A note on what this is not

This book does not diagnose you and does not prescribe for you. Doses, products and intervals described here are the ones discussed in published guidelines, not a personal prescription. Menopause care depends on your history, your examination and your test results, and the decisions belong to you and a qualified clinician. What this book is designed to do is make sure you walk into that consultation knowing what to ask.

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# Part 1. The decade nobody explained to you

Somewhere between 42 and 48, most Indian women start getting symptoms that no one in the family names. Knees that hurt on stairs. A body that wakes at 3 a.m. and will not go back to sleep. A temper that arrives before the thought does. Periods that come in fourteen days, then forty. And a chorus of advice that ranges from 'it is your age, adjust' to 'take calcium' to 'you are just stressed'.

It has a name. It is perimenopause, and it is not a personality problem.

## The one number that changes how you plan

The average Indian woman reaches natural menopause at about 46. That figure comes from the Indian Menopause Society's pan-India survey, which covered all four regions through 21 of its chapters, and it is several years earlier than the roughly 51 years reported in Western cohorts. The same survey put the average start of the perimenopausal transition at about 44.7 years. The World Health Organization's global range is 45 to 55, which means a large share of Indian women sit at the early edge of it.

Why this matters practically: the risks that menopause accelerates - bone loss, blood pressure, sugar, cholesterol - start earlier here, and they start while most women still feel perfectly well. The Indian Menopause Society's 2026 guidelines respond to exactly this with a rule they call 'Start at 35': awareness from 35, and universal screening for lifestyle disease from 40, rather than waiting for symptoms or for the Western age of 50.

> If you remember one line from this book: in India, the check-up years are your forties, not your fifties. Everything in Part 16 is built on that.

## Why nobody told you

Three things keep Indian women uninformed, and none of them is your fault.

- The symptoms were never treated as a topic. Menstruation is discussed in whispers and menopause not at all; a generation of mothers went through it while being told it was 'heat in the body'.
- The research base is thin and skewed. Most of what is taught in Indian medical colleges about menopause comes from Western trials on Western women, which is why the Indian Menopause Society wrote its own country guidelines in 2019-20 and again in 2026.
- The symptoms do not match the brochure. Indian studies consistently find that the famous symptom - the hot flush - is not the one that bothers most women here. More on that in Part 3, because it changes what you ask your doctor for.

## What changed in 2025 and 2026

You are reading this at an unusually good moment, for three reasons.

- The evidence on hormone therapy was rewritten. On 10 November 2025 the US Food and Drug Administration asked manufacturers to remove the boxed warnings about cardiovascular disease, breast cancer and probable dementia from menopausal hormone therapy labels - warnings that had frightened a generation of doctors and patients since 2002. By February 2026 the updated labels were approved for the products concerned. The Indian Menopause Society's 2026 guidelines read this alongside the 2024 Lancet series, which argued for treating menopause as part of healthy ageing rather than as a disease, without swinging to neglect.
- India started building public infrastructure. Maharashtra's health system runs dedicated menopause clinics - 580 of them had screened 31,288 women by 11 February 2026. Karnataka announced 'Ruthuthare', a statewide perimenopause and menopause programme, as the first component of what it intends to be India's first dedicated women's health policy. Tamil Nadu has been piloting similar clinics.
- Demand is visibly rising. Kantar's India in Search 2026 report recorded a 22% rise in perimenopause searches, alongside a 324% rise in searches for strength training for older adults. Women are looking for this information. Very little of it is written for them, in their context, in their language of food and family and work.

## How to use this book

Read Parts 2 to 4 first. They explain what is happening and how to get it checked properly, and they will save you from the two most common mistakes: accepting 'it is your age' as a diagnosis, and buying supplements instead of getting tested.

Then jump to whatever is actually troubling you. If it is sleep, go to Part 9. If it is your knees and your bones, Part 7. If it is sex, Part 11. If it is your boss, Part 15. Each part is written to stand alone.

Parts 13 and 14 are the doing parts - the kitchen and the twelve-week strength plan - and they are where most of the long-term benefit sits. Part 17 is the paperwork: trackers and a one-page sheet to carry to your appointment.

This is education, not a prescription. Every drug decision in here belongs to you and a qualified doctor who has examined you. What this book does is make sure you walk into that room knowing what to ask, what the guidelines actually say, and what the alternatives are when the first answer you get is 'just lose weight'.

# Part 2. What is actually happening in your body

Menopause is not an event. It is the end point of a transition that takes years, and almost everything that makes the transition hard happens before the final period, not after it.

## The four stages, in plain words

| Stage | What is happening | What you may notice |
|---|---|---|
| **Reproductive years** | Cycles are regular, oestrogen is steady | Nothing unusual |
| **Early perimenopause** | Cycles vary by 7 days or more; oestrogen swings high and low | Heavier or lighter periods, breast tenderness, disturbed sleep, irritability |
| **Late perimenopause** | Gaps of 60 days or more; oestrogen falls overall but can still spike | Hot flushes, night sweats, dryness, joint aches, low mood, brain fog |
| **Postmenopause** | Twelve consecutive months with no period, no other cause | Flushes may settle over 2-7 years; dryness and bone loss continue |

The clinical name for this staging is STRAW+10, and you do not need to memorise it. What you do need is the middle row: the years when your hormone levels are not low but unstable. That instability, not simple deficiency, is why perimenopause can feel worse than the years after it.

One important consequence: a blood test cannot tell you where you are. During perimenopause, follicle-stimulating hormone can read 12 one month and 48 the next. The Indian Menopause Society's guidance is blunt about this - menstrual irregularity is the only reliable marker of the transition, and menopause itself is diagnosed retrospectively, twelve months later.

## Why the oestrogen drop reaches so far beyond your periods

Oestrogen receptors sit in the brain, the blood vessel wall, the bone-remodelling cells, the bladder and vaginal lining, the skin, the joint cartilage and the gut. That is why one hormone falling produces a list that looks unrelated: a flush, a frozen shoulder, a urinary infection, a panic attack at a wedding, a belly that appeared in six months.

Three of those effects deserve a specific warning because they are silent.

- Bone. Loss is fastest in the two to three years around the final period, and it produces no symptom at all until a fracture. India already carries a heavy background load of low bone mass, low dietary calcium and vitamin D deficiency, so the drop lands on weaker ground.
- Blood vessels. The WHO fact sheet states it plainly: the female advantage in cardiovascular disease gradually disappears as oestrogen falls. The 2026 Indian guidelines go further, noting that cardiometabolic risk trajectories accelerate around the final menstrual period - and rate that evidence Grade A, their strongest.
- Muscle and fat distribution. Lean mass falls and visceral fat rises even when the weighing scale does not move. This is why waist measurements start climbing in women who have not changed their eating.

## The three things it is not

Menopause is not the cause of every symptom after 40. This matters, because the 'it is your age' reflex delays real diagnoses - thyroid disease, anaemia, diabetes, depression, vitamin D deficiency and early ovarian failure all present in this decade and all get missed.

It is also not an illness you must simply endure. Both the 2024 Lancet series and the 2026 Indian guidelines land on the same position from opposite directions: do not medicalise a natural transition, and do not leave a symptomatic woman untreated. The failure mode in India is overwhelmingly the second one.

And it is not the end of your body being yours. The women who do best through this decade are not the ones with the fewest symptoms. They are the ones who get measured - bone density, blood pressure, sugar, lipids, vitamin D - and then act on the numbers.

# Part 3. The symptom map - and the symptom India actually reports

Here is the finding that should reframe your whole conversation with your doctor: in Indian studies, joint and muscular pain is consistently the most reported menopausal symptom, ahead of hot flushes.

## What the Indian data shows

| Study population | Most reported symptoms |
|---|---|
| **300 postmenopausal women, Lucknow** | Joint and muscular discomfort 87%, depressive mood 70%, heart discomfort 60%, physical and mental exhaustion 60%, sleep problems 56%, hot flushes 53% |
| **Rural community, Delhi** | Sleep problems about 63%, muscle and joint pain about 59%, hot flushes about 46%, night sweats about 46%, depressed mood about 32% |
| **Outreach clinics, South Canara** | Joint and muscle pain 85% and physical exhaustion 85%; average 7.6 symptoms per woman; 53% had at least one severe symptom |
| **Rural central India** | Joint and muscle pain 78%, exhaustion 55%, hot flushes 28% |
| **398 women, Varanasi** | Joint and muscle pain 80%, depressive mood 70%, anxiety 69%, hot flushes 69%; psychological symptoms were the most often rated severe |
| **Multicentric study, 1,479 women aged 40-60** | Hot flushes 37.6% overall, of which only 2.2% severe; hysterectomy was strongly associated with flushes |
| **Systematic review of 23 Indian studies, 5,074 women** | Somatic symptoms on average 74%, psychological 63%, urogenital 51% |

Read those rows together and a pattern appears. The average Indian woman in these studies reports six to eight symptoms, led by aches, exhaustion and low mood, with flushes present in somewhere between a quarter and two-thirds depending on the sample. Western material, where flushes dominate, prepares you for the wrong thing - and prepares your doctor for the wrong thing too, which is why a woman who says 'my knees and my sleep' gets sent to an orthopaedic surgeon and a sleep tablet.

> Practical consequence: if your main complaints are joint pain, exhaustion, poor sleep and low mood, that is a completely typical Indian perimenopausal picture. Say the word perimenopause out loud in the consultation. It changes which tests get ordered.

## The full map

The table below groups the recognised symptoms by system. Use it as a checklist, not a diagnosis. Tick what applies to you, note how many days a month it happens, and take it to your appointment. Frequency matters more than the label.

| System | Symptoms | Usually starts |
|---|---|---|
| **Cycle** | Shorter or longer cycles, heavier bleeding, spotting, missed months | Early perimenopause |
| **Heat regulation** | Hot flushes, night sweats, chills, palpitations during a flush | Late perimenopause |
| **Sleep** | Waking at 2-4 a.m., difficulty falling asleep, unrefreshing sleep | Early to mid |
| **Mood and mind** | Irritability, anxiety, low mood, crying spells, loss of interest, word-finding difficulty, poor concentration | Early to mid |
| **Musculoskeletal** | Joint pain (knees, hands, shoulders), muscle aches, stiffness, frozen shoulder, new back pain | Early |
| **Energy** | Physical and mental exhaustion not explained by workload | Early |
| **Genitourinary** | Vaginal dryness, burning, pain during sex, recurrent urinary infections, urgency, leaking | Mid to late, then persistent |
| **Sexual** | Lower desire, less arousal, pain, changed orgasm | Mid to late |
| **Skin and hair** | Dryness, itching, thinning hair, more facial hair, slower healing | Mid |
| **Body composition** | Waist fat, loss of muscle tone, weight creep without diet change | Early |
| **Head and senses** | New or worse migraine, dizziness, ringing ears, dry eyes, altered taste | Variable |
| **Heart and vessels** | Palpitations, breathlessness on exertion, rising blood pressure | Mid |

## Symptom to action, in one table

If you take one page to your doctor, take this one. It converts what you are feeling into the specific thing to ask for.

| What you notice | What it most likely is | What to ask for |
|---|---|---|
| **Aches that move around; morning stiffness under an hour** | Perimenopausal joint and muscle symptom | Symptom review, DXA if due, physiotherapy rather than painkillers alone |
| **Waking at 2-4 a.m., often damp** | Night sweats driving sleep disruption | Flush treatment plus the behavioural sleep plan in Part 9 |
| **Burning or urgency when passing urine, repeatedly** | Genitourinary syndrome, not a hygiene failure | Urine culture, then a discussion about vaginal oestrogen |
| **Periods every two to three weeks, or very heavy** | Perimenopausal anovulation; exclude fibroid or polyp | Pelvic ultrasound, haemoglobin and ferritin |
| **Low mood and no interest for more than two weeks** | Depression, common in this window | A proper mood assessment, not reassurance |
| **Forgetting names, losing the thread** | Perimenopausal cognitive change, or thyroid, B12, anaemia | TSH, B12, ferritin, and a sleep review |
| **Flushes with a racing heart** | Usually a vasomotor flush; occasionally an arrhythmia | ECG if it happens at rest or with chest pain |
| **Pain during sex** | Vaginal atrophy | Local oestrogen and a lubricant, not more foreplay |
| **Waist growing while weight stays the same** | Muscle loss with visceral fat gain | Protein target and resistance training, Parts 13 and 14 |
| **Hair thinning at the parting** | Androgen change, or iron or thyroid disease | Ferritin and TSH first, then dermatology |

## The symptoms that most often get another diagnosis

Four presentations in this decade are routinely mislabelled, and each has a different fix.

- Joint pain that is blamed on age. Perimenopausal joint pain typically moves around, affects hands and knees, is worse in the morning and improves with movement. If one joint is hot, swollen and locked, that is not hormonal - get it examined.
- Palpitations that are called anxiety. A flush can come with a racing heart. But a racing heart with chest pain, breathlessness at rest, or fainting is a medical emergency, not a flush.
- Recurrent urinary infections that are treated with antibiotic after antibiotic. Falling oestrogen thins the vaginal and urethral lining and changes the bacterial balance. Low-dose vaginal oestrogen addresses the cause; repeated antibiotics do not.
- Low mood that is put down to 'empty nest' or 'hormones'. Depression is a real and treatable illness, more common in this window, and it deserves assessment rather than endurance. Cognitive behavioural therapy and antidepressants both work; so does treating the transition when symptoms are driving it.

### Rating your own symptom load

A validated tool called the Menopause Rating Scale (MRS) is used in Indian studies and takes two minutes. You do not need the official form; a simple version works for tracking. For each symptom, score 0 (none) to 4 (very severe) based on the last month, then add the scores.

| Score band | What it usually means | What to do |
|---|---|---|
| **0-4** | Minimal | Lifestyle plan in Parts 13-14, screening in Part 16 |
| **5-8** | Mild | Same, plus a discussion about local treatment if dryness is present |
| **9-15** | Moderate | Book a proper menopause consultation; discuss all options including hormone therapy |
| **16 and above** | Severe | Do not wait. Severe symptom scores plus sleep loss is the classic picture where treatment changes lives |

# Part 4. Getting a diagnosis you can trust

The single most useful skill in this decade is knowing which tests mean something and which are noise. Most Indian women are either over-tested with a random hormone panel or under-tested on everything that matters.

## What a hormone test can and cannot tell you

Follicle-stimulating hormone (FSH) rises as the ovaries stop responding. The Indian guidelines describe the bands: above 10 IU/L suggests declining ovarian function, above 20 IU/L with symptoms suggests ovarian insufficiency in the perimenopausal age group even if periods continue, and above 40 IU/L on two samples at least four weeks apart is a reliable marker of menopause or impending menopause. Anti-Mullerian hormone falls to undetectable, inhibin falls, and ovarian volume shrinks.

None of that makes FSH a good screening test in your forties, because it fluctuates. A normal result does not rule out perimenopause, and a high result in a woman still having periods does not mean she cannot conceive. The test earns its place in three specific situations: suspected premature ovarian insufficiency under 40, women who have had a hysterectomy and therefore have no periods to read, and women on hormonal contraception whose cycles are masked.

Under 40, it is different. Premature ovarian insufficiency is diagnosed on FSH above 25 IU/L on two occasions four weeks apart with four months of irregular periods, and it needs treatment, not reassurance - the 2026 guidelines are clear that these women should receive hormone therapy at least until the average age of natural menopause. If you are under 40 with stopping periods, do not accept 'stress' as the answer.

## The tests that actually matter at 40 and after

| Test | Why it is ordered | How often |
|---|---|---|
| **Blood pressure** | Rises through this decade; the leading modifiable risk | Every visit, or home log monthly |
| **HbA1c or fasting glucose** | Diabetes risk climbs with visceral fat and falling oestrogen | Every 1-3 years, annually if borderline |
| **Lipid profile** | LDL and triglycerides tend to rise after menopause | Every 1-2 years |
| **Haemoglobin and ferritin** | Heavy perimenopausal bleeding causes real iron deficiency, often missed | Now, and again if bleeding is heavy |
| **TSH** | Thyroid disease mimics every menopausal symptom | Once, then as indicated |
| **Vitamin D** | Deficiency is widespread in Indian women; affects bone and muscle | Once, then after 3-4 months of treatment |
| **Serum calcium** | Baseline before supplements; also screens parathyroid problems | Once |
| **DXA bone density** | The only way to find low bone mass before a fracture | At menopause, or earlier with risk factors |
| **Mammogram** | Breast screening from 40, or earlier with family history | Every 1-2 years, per your doctor |
| **Pap or HPV test** | Cervical screening; VIA is less reliable after menopause | Every 3-5 years per protocol |

## The heavy-bleeding exception

Perimenopausal bleeding can be heavy, but heavy is not automatically normal. The Indian guidelines and every gynaecology protocol agree on the rule: bleeding that soaks a pad an hour for two hours, bleeding between periods, bleeding after sex, or any bleeding at all after twelve months of none, needs examination and usually a transvaginal ultrasound and endometrial sampling. Do not let this wait for a 'convenient month'.

### How to open the conversation

Indian consultations are short, and 'I have many problems' gets you three minutes. Lead with the pattern and the ask, in that order.

> Script: 'Doctor, I am 46. My periods have become irregular over the last eight months, and I am waking at night with sweating, my knees ache, and my mood is low. I think I am perimenopausal. I would like my blood pressure, sugar, haemoglobin, ferritin, thyroid and vitamin D checked, and I want to discuss whether hormone therapy is appropriate for me.'

Then ask the three questions that decide everything: What is my bone density and should I have a DXA scan? What are my cardiovascular risk numbers? And if we do not use hormones, what is the plan for the next twelve months?

If you are told 'it is your age, it will pass', you are entitled to one more question: 'Which of my symptoms will pass, and what will we do about the ones that will not?' Bone loss and vaginal dryness do not pass. They progress.

# Part 5. Hormone therapy, honestly

No topic in women's health carries more fear and more misinformation than this one. So let us start with what the guidelines and regulators now say, and then work through the parts that actually matter to you: whether it suits you, what it involves, what it costs, and what the real risks are in numbers you can hold in your head.

## What the 2026 position actually is

The Indian Menopause Society's Clinical Practice Guidelines for Menopause 2026, published as a supplement to the Journal of Mid-life Health, reviewed the North American Menopause Society 2023 statement, the International Menopause Society 2024 recommendations, NICE guideline NG23 as updated in 2024, and the European Menopause and Andropause Society 2023 position. The consensus across all of them is narrower and more practical than the internet suggests.

- Menopausal hormone therapy remains the most effective treatment for vasomotor symptoms - flushes and night sweats.
- It works best when started within ten years of the final menstrual period, or before 60 - the 'timing hypothesis'.
- Low-dose and transdermal (patch or gel) regimens are preferred where there is clot risk, because they avoid the first pass through the liver.
- There is no mandatory age at which you must stop. Continuation is a yearly conversation, not a countdown.
- For women who cannot or will not take hormones, non-hormonal drug options and cognitive behavioural therapy are genuine alternatives, not consolation prizes.

On 10 November 2025 the US FDA moved in the same direction, asking manufacturers to remove the boxed warnings on cardiovascular disease, breast cancer and probable dementia from hormone therapy labels, and by 12 February 2026 the revised labels were approved for the products affected. That does not mean hormones are risk-free. It means the warning language had stopped matching the evidence, and the regulator said so.

## The 2002 story you have probably heard, corrected

The Women's Health Initiative was a large American trial of older women, average age about 63, most of them more than a decade past menopause. It found that one particular combined regimen increased breast cancer risk and did not protect the heart. The results were published in 2002 and turned into a blanket fear that has lasted a generation.

Three things were lost in the telling. The women studied were not the women now asking for help - they were older and mostly asymptomatic. The oestrogen used was conjugated equine oestrogen with a synthetic progestin, not the estradiol and micronized progesterone used in modern practice. And the finding of harm applied to starting hormones long after menopause, which is precisely the opposite of the timing hypothesis.

The 2024 Lancet series on menopause and the 2026 Indian guidelines both treat this as settled history. The current framing is: for a woman under 60 or within ten years of menopause with bothersome symptoms, the benefits of hormone therapy generally outweigh the risks, and the decision should be individual rather than categorical.

## The real risk numbers, in context

Absolute risk is the only honest way to discuss this. A relative risk of 1.3 sounds alarming and can mean three extra cases per thousand women per year. When your doctor quotes a percentage, ask: per thousand women, over how many years?

| Concern | What the evidence supports | What it means for you |
|---|---|---|
| **Breast cancer** | Risk rises with combined oestrogen plus progestogen and with years of use; the absolute increase is small in the 50-59 age group and larger the longer you continue | Reassess annually; discuss breast density and screening interval; stop if you have had breast cancer |
| **Blood clots** | Increased with oral oestrogen; minimal with transdermal oestrogen at standard doses | If you have clot risk factors - smoking, BMI above 30, previous clot, immobility - ask for transdermal |
| **Stroke** | Not increased in women starting under 60 at standard doses; increased in older women starting late | Another reason timing matters more than duration |
| **Heart disease** | No protection, and no harm when started in the window; the old 'cardioprotection' claim was wrong | Do not take hormones for your heart; take them for symptoms and bone, and manage the heart separately |
| **Bone** | Reduces fracture risk at spine and hip; approved as a prevention option in appropriate women | A real benefit for women with low bone mass who also have symptoms |
| **Endometrium** | Unopposed oestrogen raises endometrial cancer risk; adding adequate progesterone for 12-14 days a month, or continuously, prevents it | Never take oestrogen alone if you have a uterus |

## What a regimen looks like

This is the part nobody explains before you reach the pharmacy counter. Hormone therapy is not one drug; it is a choice of oestrogen, a route, and - if you have a uterus - a progestogen to protect the lining.

| Component | Common options | Notes |
|---|---|---|
| **Oestrogen, oral** | Estradiol 1-2 mg daily | Simple, cheap, widely available; passes through the liver |
| **Oestrogen, transdermal** | Patch 25-100 micrograms daily, or gel | Preferred with clot or triglyceride risk, and often for migraine; adherence is the issue in Indian heat |
| **Progestogen** | Micronized progesterone 200 mg for 12-14 days a month, or 100 mg daily continuous; or a levonorgestrel intrauterine system | Micronized progesterone is better tolerated and is what the 2026 guidelines point to; the intrauterine system also controls heavy bleeding |
| **Combined products** | Continuous combined oestrogen plus progestogen | For women more than a year past menopause; expect irregular spotting for the first months |
| **Local vaginal oestrogen** | Estradiol 25 micrograms daily, estriol cream 0.5 mg, conjugated oestrogens 0.3-1.25 mg | For dryness, pain and recurrent urinary symptoms; systemic absorption is minimal and progesterone is not needed |
| **Tibolone** | 2.5 mg daily | A synthetic option used in India; discuss liver and cancer history with your doctor |

The 2026 guidelines make two specific technical points worth knowing. First, transdermal oestrogen may be preferred because it maintains steadier blood levels and improves sleep quality in women with significant flushes - a Grade B recommendation, meaning moderate evidence. Second, micronized progesterone may carry additional benefit over synthetic progestins. If your doctor offers only one standard combination, it is reasonable to ask about these.

## What the first three months feel like

Flushes usually begin to ease within two to four weeks and often halve by three months. Sleep improves next, then mood and joint aches. Vaginal dryness responds more slowly and often needs local treatment alongside, because systemic oestrogen at standard doses does not always reach the vaginal tissue in sufficient concentration.

Expect some bleeding in the first three to six months on a continuous combined regimen. That is usually the lining settling. Report any bleeding after six months, and any bleeding that is heavy or unexpected on a cyclical regimen.

If nothing has changed by eight to twelve weeks at an adequate dose, the answer is not to keep waiting. It is to review the dose, the route, and the diagnosis - because a third of women labelled 'hormone resistant' turn out to have been under-dosed, and a proportion were never having hormonal symptoms at all.

### When hormones are not the answer

There are absolute contraindications, and they are not negotiable: a history of breast cancer, current or past venous thromboembolism on oral therapy, active liver disease, undiagnosed abnormal genital bleeding, and a history of endometrial cancer or certain other oestrogen-dependent tumours. There are also situations where the balance tips against: very high stroke risk, uncontrolled hypertension, and personal preference after a genuine discussion.

None of those leave you without options. Part 6 covers the non-hormonal routes in detail, including prescription drugs, cognitive behavioural therapy, and local treatments that remain safe in most women who cannot take systemic hormones.

### Practicalities in India

Generic oral estradiol and micronized progesterone are inexpensive here compared with imported brands, which is why cost is rarely the real barrier. Transdermal patches and gels cost more, and stock at neighbourhood chemists is inconsistent - ask the chemist to order, or ask your doctor about a gel, which is easier to store in Indian summer than a patch that peels.

Keep a written record: product, dose, start date, and the review date. Take it to every appointment, including the ones with an orthopaedic surgeon or physician who will ask what you are taking. And never stop abruptly on the advice of a well-meaning relative; taper and review with the prescriber.

### A decision worksheet

Hormone therapy decisions go wrong in two directions: women who need it never offered it, and women who should not take it given it without a proper history. Answer these eight questions before the consultation, and you will have covered what a good doctor asks.

| Question | Why it decides something |
|---|---|
| **Do my symptoms affect sleep, work or daily life on most days?** | Severity, not diagnosis, is what justifies treatment |
| **Am I within ten years of my last period, or under 60?** | This is the window in which the benefit-to-risk balance is most favourable |
| **Do I still have a uterus?** | If yes, you need a progestogen with oestrogen to protect the lining |
| **Any history of breast cancer, clot, stroke, liver disease, or unexplained bleeding?** | These are the contraindications that change the answer entirely |
| **Do I smoke, carry a BMI above 30, or have migraine with aura?** | Points towards a transdermal route rather than oral |
| **Do I have low bone mass, or a parent with a hip fracture?** | Adds a bone benefit to the calculation |
| **Can I commit to an annual review and keep a written record?** | Continuation is a yearly conversation, not a lifetime default |
| **If hormones are unsuitable, what is my plan B?** | Non-hormonal drugs, CBT and local treatment, chosen in advance |

# Part 6. The non-hormonal routes, ranked by evidence

If hormones are contraindicated, unwanted, or simply not your first choice, you are not left with calcium and patience. There is a hierarchy of options here, and it is worth knowing which ones have trials behind them.

## Prescription options

| Option | What it does | Who it suits | Watch for |
|---|---|---|---|
| **SSRIs and SNRIs (low-dose paroxetine, escitalopram, venlafaxine)** | Reduce flush frequency and severity by stabilising hypothalamic temperature regulation | Women who also have low mood or anxiety, or cannot take oestrogen | Nausea early on; sexual side effects; do not stop abruptly |
| **Gabapentin or pregabalin** | Reduces flushes, especially night sweats | Women with disturbed sleep | Drowsiness; dose at night; kidney dose adjustment |
| **NK3 receptor antagonists** | Block the brain pathway that destabilises temperature control | Moderate to severe flushes, no hormones | Newer and expensive; ask about availability in India |
| **Clonidine** | Older option, modest effect | Rarely first choice now | Dry mouth, low blood pressure |
| **Local vaginal oestrogen** | Restores vaginal and urethral lining | Almost all women with dryness, pain or recurrent urinary symptoms | Not for women with undiagnosed bleeding; safe with most breast cancer histories after oncology discussion |
| **Oxybutynin** | Reduces flushes; also treats urgency | Women with both flushes and bladder urgency | Dry mouth, constipation |

Two of those rows deserve an explanation, because they are the newest and the most asked about.

Elinzanetant received US FDA approval on 24 October 2025 for moderate to severe menopausal hot flushes, based on three trials involving 1,423 participants, taken as two capsules at bedtime. Fezolinetant, the first drug in the same class, was approved in 2023. Both are genuinely new mechanisms rather than recycled antidepressants. Availability and price in India vary by city and by hospital pharmacy, so the honest question to ask is not 'is it approved' but 'can I get it here, and what will it cost me per month'.

Cognitive behavioural therapy is the other option people skip because it sounds like a consolation. It is not. Trials of CBT for hot flushes and night sweats show meaningful reductions in bother, and the effect holds after the course ends. If your flushes are moderate but your distress is high - which is common in women who cannot take hormones - this is the intervention with the best evidence-to-risk ratio on the list.

## The supplement aisle, audited

This is where most Indian women spend money first and get relief last. The pattern is consistent: a product makes a claim in a category where regulation is light, and the trial evidence is either absent, small, or negative.

| Product | What the evidence says | Verdict |
|---|---|---|
| **Black cohosh** | Cochrane reviews have repeatedly found insufficient evidence of benefit over placebo, with rare liver concerns | Not worth the money |
| **Soy and red clover isoflavones** | Small, inconsistent reductions in flush frequency; better in some subgroups | Modest at best |
| **Evening primrose oil** | No effect on flushes in trials | Skip |
| **DHEA supplements** | No reliable benefit for symptoms; long-term safety unclear | Skip |
| **Compounded 'bioidentical' hormones** | Not standardised, not regulated for purity or dose, and marketed on the word bioidentical which has no legal meaning in India | Avoid; use approved products |
| **Calcium and vitamin D** | Genuinely important for bone, independent of symptoms | Take if intake is low |
| **Magnesium, ashwagandha, 'hormone balance' powders** | Little or no trial evidence for menopausal symptoms specifically | Optional, not treatment |
| **Yoga and paced breathing** | Modest benefit for sleep, stress and bother; small effect on flush frequency | Worth doing, not a substitute |

The rule that saves money: nothing in the supplement aisle treats vaginal dryness, and nothing in it protects bone as effectively as adequate calcium, vitamin D, protein and resistance exercise. Spend there.

# Part 7. Bone - the quiet emergency

Osteoporosis produces no symptoms until it produces a fracture, and by then you have lost the years in which it was easiest to prevent. This is the part of menopause where Indian women are worst served by the general advice they receive.

## Why the Indian starting point is lower

Three factors stack on top of each other. Indian diets are typically low in calcium - one study of rural South Indian postmenopausal women found an average intake of about 420 mg a day, roughly a third of what is recommended. Vitamin D deficiency is widespread, and in the same study more than half the women were deficient. And Indian women tend to reach menopause earlier, which means bone loss starts earlier and has more years to run.

The consequence shows up in the numbers from that rural South Indian cohort: osteoporosis in about 22% of women at the femoral neck and about 39% at the lumbar spine. A separate analysis estimated that India accounted for a large share of the world's osteoporotic population, with roughly one in five Indians over 50 affected. These are not rare outcomes at the tail of a distribution. They are the middle of it.

## The calcium argument Indian experts are still having

You will meet two numbers, and it helps to know why.

- ICMR-NIN, in its 2020 recommended dietary allowances and restated in the 2024 Dietary Guidelines for Indians, sets 1,000 mg a day for adult women and 1,200 mg a day for postmenopausal women.
- The Indian Menopause Society's 2019-20 osteoporosis guidelines retained the older 800 mg recommendation, on the grounds that Indian trial data on higher intakes was thin, and that the benefit of raising intake is greatest in women who are genuinely deficient.

The practical reading, which both positions support: get as much as you can from food, aim for somewhere in the 800 to 1,000 mg range, and supplement the gap rather than starting from a tablet. A woman eating curd twice a day, a ragi roti, and til chutney is not in the same position as a woman eating none of those. Part 13 gives you the arithmetic.

One caution that comes directly from the guidelines: do not take high-dose calcium on the assumption that more is better. Above roughly 1,200 to 1,500 mg a day there is no additional bone benefit, and there are questions about kidney stones and cardiovascular calcification. Split doses above 500 mg, because absorption falls with large single doses.

## Vitamin D, in an Indian context

The paradox of Indian vitamin D is that deficiency is common in a country with abundant sun. Skin pigmentation, indoor work, purdah and clothing practices, sunscreen habits, pollution, and the habit of avoiding midday sun all reduce synthesis. The guidelines' practical prescription is exposure of about 15-30% of body surface for around 30 minutes between 10 a.m. and 3 p.m., which is culturally difficult for many women and needs a conversation rather than a lecture.

Where sun exposure is limited, supplements are the route. A common approach is 800-1,000 IU daily for maintenance, with higher corrective doses for documented deficiency, prescribed and rechecked. Test before you treat at high dose, and recheck after three to four months - otherwise you are guessing.

## The scan that tells you the truth

A DXA scan measures bone mineral density at the spine and hip and reports a T-score: the number of standard deviations from a young adult peak. A T-score of -1.0 to -2.5 is osteopenia, low bone mass. At -2.5 or below, it is osteoporosis. A fragility fracture - a break from a fall from standing height or less - is osteoporosis regardless of the number.

The Indian Menopause Society advises a DXA scan for postmenopausal women, and earlier for women with risk factors. In practice, that means: get one at menopause, or now if you are already past it and have never had one. DXA machines are unevenly distributed in India - roughly a fraction of the density seen in high-income

countries - so if you are in a smaller town, ask at the district hospital or a private centre and travel for it once. One scan establishes your baseline for a decade.

| Risk factor | Why it matters |
|---|---|
| **Previous fracture after 40** | The strongest single predictor of the next one |
| **Parent with hip fracture** | Genetic contribution to peak bone mass |
| **Low BMI, or weight loss in the last year** | Less mechanical load and less fat-derived oestrogen |
| **Long steroid use, or inhaled steroids at high dose** | Direct bone toxicity |
| **Smoking, more than two drinks a day** | Both reduce bone formation |
| **Early menopause, or ovaries removed** | More years without oestrogen |
| **Thyroid disease, diabetes, rheumatoid arthritis, malabsorption** | Secondary causes that need separate treatment |
| **Recurrent falls, poor vision, sedative use** | Fracture risk is fall risk as much as bone density |

## What actually protects bone

Four things, in order of effect: resistance and impact exercise that loads the skeleton; adequate calcium and vitamin D; protein, so that the muscle pulling on the bone stays; and, for women with confirmed osteoporosis or high fracture risk, drug treatment - bisphosphonates being the usual first line, with hormone therapy as a legitimate option in women who also have symptoms and are within the treatment window.

What does not work: walking alone. Walking is excellent for the heart and the mood, but it is not enough loading to rebuild a hip. The exercise plan in Part 14 exists specifically because the bone needs to be pushed against gravity with progressive resistance.

And the part nobody mentions: fall prevention is half the job. Loose rugs, a bathroom without a grab bar, poor lighting on the stairs, blood pressure medication that makes you dizzy on standing, and untreated vision all convert a strong bone into a fracture.

# Part 8. Heart and metabolism - the risk that moves in quietly

Cardiovascular disease is the leading cause of death in Indian women, and the gap between male and female risk narrows after menopause. The WHO fact sheet states the mechanism simply: the female advantage in cardiovascular disease gradually disappears as oestrogen falls. The 2026 Indian guidelines add the timing - risk trajectories accelerate around the final menstrual period, and rate that Grade A.

The reason this deserves its own chapter is that none of it feels like anything. There is no symptom for a rising LDL, a fasting sugar of 108, or a blood pressure of 138/88. The only way to catch it is to measure it in your forties, which is exactly what most Indian women do not do.

## What changes after menopause

| Measure | What typically happens | What to do |
|---|---|---|
| **LDL cholesterol** | Rises, often by 10-15% in the transition | Lipid profile every 1-2 years; diet and statin discussion if indicated |
| **Triglycerides** | Rise with visceral fat and refined carbohydrate | Cut refined carbs and alcohol; move daily |
| **Blood pressure** | Tends to rise; salt sensitivity increases | Home log; keep under 120/80 where possible |
| **Fasting glucose and HbA1c** | Insulin resistance increases with visceral fat and lost muscle | HbA1c every 1-3 years; annually if borderline |
| **Waist circumference** | Rises even at stable weight | Track waist monthly, not just weight |
| **Muscle mass** | Falls about 1% a year after 40 without resistance training | Part 14, twice a week minimum |

## The waist number, not the scale number

Body weight is a poor guide in this decade because muscle is being replaced by fat at the same weight. Waist circumference is the better proxy for the visceral fat that drives insulin resistance. For Indian women, a waist above 80 cm is the threshold used in national guidance for increased metabolic risk, and above 88 cm for substantially increased risk. Measure at the narrowest point between the lowest rib and the hip bone, at the end of a normal breath out, without pulling the tape tight.

If your waist has grown but your weight has not, you have not failed at anything. You have lost muscle and gained fat, which is the default trajectory without resistance training and adequate protein. It is reversible at 45 and at 55; it becomes much harder at 65, which is why the strength plan in Part 14 is not optional content.

## The five numbers to know by heart

- Blood pressure - measured at home, twice, sitting, after five minutes rest.
- HbA1c - one number for the last three months of sugar.
- LDL cholesterol - the number that responds to diet, exercise and statins.
- Waist circumference in centimetres.
- Waist-to-height ratio - waist divided by height, both in the same units. Above 0.5 is the simple flag used widely in Indian screening.

Write them in Part 17 each time you test. Trends beat single readings, and a doctor who sees four years of numbers makes better decisions than one who sees one.

### What actually moves these numbers

Three interventions have large effects and are unglamorous. Reducing added salt towards the ICMR-NIN recommendation of under 5 g a day, which in Indian cooking means attacking pickles, papad, packaged namkeen, and the extra salt at the table as much as the salt in the dal. Replacing refined cereal with whole grains and millets at the same total quantity, and keeping cereals to about 45% of energy as the 2024 guidelines advise. And 150 to 300 minutes a week of moderate activity plus two sessions of resistance training, which is the WHO prescription and also the one that preserves muscle.

What does not move them: hormone therapy, which neither protects nor harms the heart when started in the window, and should never be taken for cardiac reasons. This is one of the few places where the old marketing and the current evidence diverge sharply, and it is worth being clear about it.

# Part 9. Sleep - the symptom that amplifies all the others

Sleep is where perimenopause does its worst damage, because sleep loss makes everything else worse: mood, pain tolerance, blood sugar, appetite, patience, and the ability to cope with a hot flush at a wedding.

The pattern is specific and recognisable. You fall asleep fine and wake between 2 and 4 a.m., sometimes drenched, sometimes just awake with your mind running. Or you sleep through but wake unrefreshed. In Indian studies, sleep problems are reported by roughly 56-63% of menopausal women, which makes this the second most common complaint after aches.

## The three causes, separated

| Cause | Signature | First move |
|---|---|---|
| **Vasomotor - night sweats** | Waking hot and damp, needing to change clothes or throw off the quilt | Cool the room, cotton layers, treat the flushes |
| **Hormonal sleep disruption** | Waking at 2-4 a.m. without sweating; lighter sleep architecture | Review hormone therapy if symptoms are otherwise significant |
| **Anxiety or depression** | Lying awake with thoughts; early waking with low mood in the morning | Assess and treat the mood disorder directly |
| **Sleep apnoea** | Snoring, witnessed pauses, morning headache, daytime sleepiness | Sleep study; risk rises sharply after menopause |
| **Behavioural** | Late caffeine, late screens, alcohol, irregular bedtimes, long daytime naps | Fix the basics before anything else |

## The four-week sleep reset

Before any tablet, run this for four weeks. It is the core of cognitive behavioural therapy for insomnia, which outperforms sleeping pills in the long run.

- Fix the wake time, seven days a week, including weekends. This one lever does more than any other.
- Get out of bed if you are awake and frustrated for more than about twenty minutes. Sit in dim light and read something dull. Return when sleepy. This retrains the bed as a place for sleep.
- Cut caffeine after 2 p.m. - including the filter coffee at 5 p.m. that feels like tradition.
- Stop alcohol as a sleep aid. It shortens the time to sleep and then fragments the second half of the night, which is precisely the part that is already broken.
- Cool the room and the body: a light cotton quilt, a fan directly on you, a cool shower before bed if you flush at night.
- Get bright light in the first hour after waking, ideally outdoors. Morning light anchors the body clock that perimenopause has destabilised.
- Keep a two-line sleep diary: bedtime, wake time, number of wakings, and one word for how you felt. Four weeks of this is more useful to a doctor than any description in the room.

If night sweats are the driver, treat the flushes - that is Part 5 and Part 6. If anxiety is the driver, treat the anxiety. Sleeping pills prescribed for a hormonal problem are the most common wrong turn in this decade, and dependence develops faster in women over 45.

One important clinical note from the 2026 guidelines: hormone therapy is not indicated as a primary treatment for insomnia. It helps sleep when flushes are waking you, and it does not fix insomnia that exists on its own. That distinction decides whether the answer is a hormone, a behaviour change, or both.

# Part 10. Mood, anxiety and the fog

The mood symptoms of perimenopause are the ones most often dismissed and most often misattributed. Indian data is unambiguous about their weight: in the Lucknow series, depressive mood was reported by 70%; in Varanasi, psychological symptoms were the ones most frequently rated severe, at 52%. A systematic review of 23 Indian studies put psychological symptoms at an average of 63%.

That is not a small side effect. It is a central feature.

## Why the mood shifts

Three mechanisms, all real. Oestrogen modulates serotonin and dopamine systems, so its fluctuation changes mood regulation directly. Sleep loss degrades emotional control measurably. And the life events that cluster in this decade - children leaving, parents needing care, a marriage that has settled into a routine, a career plateau, a body that has changed - are genuinely difficult, independent of hormones.

The clinical picture that should prompt assessment rather than endurance: low mood most days for two weeks or more, loss of interest in things you used to enjoy, changed sleep and appetite, guilt or worthlessness, and thoughts that life is not worth it. That last one is not a symptom to be managed with willpower. If it appears, speak to a doctor or a mental health professional now.

## What helps, in order

- Sleep. Treat it first, because mood rarely improves on five hours a night.
- Movement. Regular moderate exercise has effect sizes comparable to medication for mild to moderate depression, and it is additive with everything else here.
- Talking therapy. Cognitive behavioural therapy has good evidence for perimenopausal low mood and anxiety, and it is available in India through online platforms and hospital psychiatry departments.
- Treat the transition. If flushes and sleep loss are driving the mood, controlling them often lifts the mood without any antidepressant.
- Medication. SSRIs and SNRIs work, and several of them also reduce flushes - a rare double benefit. Discuss sexual side effects and the tapering plan before you start.

## Brain fog, and what it really is

Word-finding difficulty, walking into a room and forgetting why, losing the thread of a sentence, slower recall of names. This is one of the most frightening symptoms because it raises the question of dementia, and one of the most reliably reported in perimenopause.

The current understanding is reassuring. Objective performance on memory tests in midlife shows only small changes, and studies generally find the effect is transient, improving after the transition. What makes it feel much worse than it measures is sleep debt, anxiety, and the sheer cognitive load of this decade. It is also worth ruling out the treatable mimics: thyroid disease, anaemia, vitamin B12 deficiency, untreated sleep apnoea, and depression.

Practical countermeasures that work: write things down instead of holding them, reduce multitasking to single tasks where possible, protect sleep, and get the blood tests in Part 4 done so you are not carrying an untreated thyroid or B12 problem on top of a hormonal one.

> If a relative tells you 'it is all in your head', they are half right - oestrogen receptors are in the brain. That is precisely why it is real, treatable, and not something to be ashamed of.

# Part 11. Dryness, bladder, sex and the pelvic floor

Genitourinary syndrome of menopause is the part of this transition that does not go away on its own, does not respond to lifestyle, and is almost never discussed in Indian households. It affects a majority of postmenopausal women to some degree, and it is one of the most treatable problems in the whole book.

The old name was vulvovaginal atrophy. The newer name, genitourinary syndrome of menopause, exists because the problem is not just dryness - it involves the urethra and bladder too. Falling oestrogen thins the vaginal lining, reduces lubrication and elasticity, raises the pH, and changes the bacterial population. The result: dryness, burning, itching, pain during sex, and a striking increase in urinary urgency, frequency and recurrent infections.

## Why nothing you were told works

Systemic hormone therapy at standard doses does not reliably fix vaginal symptoms, because the tissue needs a higher local concentration than the rest of the body does. This is why women on tablets or patches for flushes still have pain with sex, and conclude that nothing works.

Local oestrogen is the answer, and it is one of the safest treatments in the formulary. A Cochrane review found all licensed local oestrogen preparations equally effective. The 2026 Indian guidelines list the standard doses: conjugated equine oestrogens 0.3-1.25 mg a day, estradiol 25 micrograms a day, estriol cream 0.5 mg a day, chosen by severity. Absorption into the bloodstream is minimal, progesterone is not needed, and it can be used long term - in fact it needs to be, because symptoms return when you stop.

| Option | How it is used | Notes |
|---|---|---|
| **Vaginal estradiol tablet or cream** | Daily for two weeks, then twice weekly | The mainstay; needs a prescription |
| **Vaginal moisturiser** | Two to three times a week, not for intercourse | Non-hormonal, available over the counter, helps maintenance |
| **Lubricant** | At the time of sex | Water-based or silicone; avoid petroleum jelly and oil-based products with condoms |
| **Pelvic floor exercises** | Daily, correct technique | Improves urgency, leaking and sensation |
| **Ospemifene** | Oral, for moderate to severe dryness and pain | Prescription; a non-hormonal-receptor option for some women |

## The bladder, specifically

Recurrent urinary tract infections after menopause are usually not a hygiene failure. They are a tissue problem. If you have had three or more in a year, ask about vaginal oestrogen rather than another antibiotic course, and ask for a urine culture before treatment rather than after. The behavioural basics matter too: adequate water, treat constipation, empty the bladder after sex, and do not cut fluids out of fear of leaking - concentrated urine irritates the bladder and worsens urgency.

## Sex, honestly

Desire often falls, arousal takes longer, and intercourse may hurt. Three separate things are being conflated here, and they have separate fixes. Pain is a tissue problem - treat it with local oestrogen and lubricant. Arousal difficulty is partly vascular and partly about context and safety. Desire is the most complex of the three, and the least responsive to a single intervention.

What helps in practice: treat the pain first, because pain extinguishes desire faster than anything else. Expand what counts as sex beyond intercourse. And talk about it with a partner somewhere other than the bedroom at 11 p.m. Your gynaecologist has this conversation regularly - you will not be the first woman to raise it, even if you are the first in your family.

Two safety points Indian women rarely hear: pregnancy is still possible during perimenopause, so contraception is needed until twelve consecutive months without a period; and sexually transmitted infections,

including HIV, remain possible after menopause, because the thinning vaginal wall makes small tears more likely.

### When to ask for a referral

Most of this is handled by a gynaecologist. Three situations deserve a urogynaecologist or a pelvic floor physiotherapist: leaking that happens with coughing, laughing or lifting; a feeling of something coming down, which is pelvic organ prolapse and is common after vaginal deliveries and with age; and pain that has not improved after three months of local oestrogen and lubricant. All three are treatable, and none of them require you to live with it. Prolapse in particular is managed well with pelvic floor physiotherapy and, where needed, a ring pessary - a simple silicone device fitted in clinic that many women use for years instead of surgery.

# Part 12. Body, skin, hair and joints

## Weight that arrives without permission

The typical pattern in this decade is a gain of a few kilos concentrated at the waist, without a change in eating. Three things drive it: falling oestrogen shifts fat storage from hips and thighs to the abdomen; muscle loss lowers resting energy expenditure; and sleep loss raises appetite-driving hormones and cravings for refined carbohydrate.

What works is not a crash diet, which accelerates muscle loss and makes the next year harder. What works is protein at every meal, resistance training twice a week, keeping refined cereal and liquid sugar low, and a modest calorie deficit if weight loss is the goal. The 2024 ICMR-NIN guidelines are useful here: cereals capped at about 45% of energy, added sugar under 5% of energy - roughly 20 to 25 g a day - visible fat and oil at 25 to 30 g a day, salt under 5 g, and half the plate as vegetables and fruit.

## Muscle, the organ you are actually losing

Muscle is the tissue that keeps you independent at 70. It is also the tissue that menopause attacks fastest, and the tissue that Indian women are least encouraged to build. The number to hold on to is protein: ICMR-NIN's recommended daily allowance for adults is 0.83 g per kg of body weight, which for a 60 kg woman is about 50 g a day. That is a floor for survival, not a target for preserving muscle through a hormonal transition; international groups working on ageing recommend 1.0 to 1.2 g per kg for older adults, and Indian diets routinely fall short even of the lower figure because the day is built on cereal.

The 2024 guidelines make a related point that surprises people: obtain protein from a combination of foods and avoid protein supplements to build muscle. Dal, curd, paneer, eggs, fish, chicken, soya and nuts do the job. A whey scoop is a convenience, not a strategy.

Protein also matters for bone, because bone remodels in response to the pull of muscle. And it matters for skin and hair, which is where the visible complaints come from.

## Skin and hair

Collagen production falls with oestrogen, so skin thins, dries and loses elasticity; hair thins at the crown and parting while facial hair may coarsen. Both are normal, both are slow, and neither is a sign of neglect.

- Skin: a gentle cleanser, a moisturiser with ceramides, sunscreen every day - pigmentation is the fastest visible ageing in Indian skin and sun is the main driver - and a retinoid if you want an active ingredient with real evidence. Skip the expensive 'hormone-balancing' creams.
- Hair: treat iron deficiency and thyroid disease first, because they cause far more hair loss than oestrogen does. Then discuss topical minoxidil with a dermatologist if the pattern is typical.
- Nails and eyes: dryness increases; lubricating eye drops and a dental check every six months matter more than they sound.

## Joints, and the pain that is not arthritis

Remember Part 3: joint and muscle pain is the single most reported symptom in Indian studies, at 70-87% depending on the sample. The typical pattern is aches that move around, worse in the morning and after rest, better with movement, affecting knees, hands, shoulders and the lower back. Frozen shoulder is markedly more common in this decade.

What helps: keep moving, load the joint gently and progressively, build the surrounding muscle, manage sleep, and treat the transition if symptoms are widespread. What does not help: complete rest, which stiffens the joint and accelerates muscle loss, and long-term painkillers without a diagnosis.

Get it examined if a joint is swollen, hot, red, or locking, if pain wakes you at night consistently, or if morning stiffness lasts more than an hour. Those are features of inflammatory arthritis, which is a different disease and needs a rheumatologist, not calcium.

# Part 13. The Indian kitchen, rebuilt for this decade

You do not need a new cuisine. You need three changes to the one you already cook: protein at every meal, calcium built into the daily pattern rather than taken as a tablet, and less refined cereal at the centre of the plate. Everything below is food your family already eats.

## The three targets

| Target | Number | Where it comes from |
|---|---|---|
| **Protein** | 1.0-1.2 g per kg body weight per day | ICMR-NIN sets 0.83 g/kg as the RDA for adults; international ageing guidance recommends 1.0-1.2 g/kg to protect muscle in midlife and beyond |
| **Calcium** | 800-1,200 mg a day from food, supplement the gap | ICMR-NIN: 1,000 mg for adult women, 1,200 mg postmenopausal. IMS 2019-20 retained 800 mg pending Indian data |
| **Fibre** | 25-30 g a day | Whole grains, dal, vegetables, fruit, seeds - the 2024 guidelines put half the plate here |

For a 60 kg woman, 1.0-1.2 g per kg is 60-72 g of protein a day. That sounds like a lot until you see it spread across four eating occasions of 15-20 g each. The typical Indian vegetarian day delivers 35-45 g, mostly at lunch and dinner, which is why muscle goes.

## Calcium in food you already eat

Values are approximate, drawn from standard Indian food composition tables, and meant for planning rather than precision. Cooking, brand and portion size move them by 10-20%.

| Food | Portion | Calcium (approx.) | Protein (approx.) |
|---|---|---|---|
| **Milk, cow** | 200 ml glass | 240 mg | 6.6 g |
| **Curd** | 150 g katori | 225 mg | 4.5 g |
| **Paneer** | 50 g | 105 mg | 9 g |
| **Ragi flour** | 30 g (2 rotis) | 103 mg | 2 g |
| **Sesame seeds (til)** | 1 tbsp, 10 g | 145 mg | 1.8 g |
| **Methi leaves** | 100 g | 395 mg | 4 g |
| **Amaranth (chaulai) leaves** | 100 g | 397 mg | 4 g |
| **Drumstick leaves** | 100 g | 440 mg | 6 g |
| **Curry leaves** | 10 g | 83 mg | 0.6 g |
| **Spinach** | 100 g | 73 mg | 2 g |
| **Almonds** | 10 nuts, 12 g | 28 mg | 2.6 g |
| **Rajma, cooked** | 150 g katori | 90 mg | 12 g |
| **Whole chana, cooked** | 150 g katori | 85 mg | 11 g |
| **Soybean chunks, dry** | 30 g | 72 mg | 15 g |
| **Milk powder** | 20 g | 270 mg | 7 g |
| **Jaggery** | 10 g | 8 mg | 0 g |

Notice what the table says. Ragi, til, methi and drumstick leaves are among the cheapest calcium in India, and they are also the foods that have quietly left urban kitchens. A tablespoon of til in chutney, one ragi roti, and a methi sabzi twice a week moves you further than an imported supplement.

Two practical notes. Plant calcium comes with oxalates and phytates that reduce absorption, so pair greens with a source of vitamin C - lemon, amla, tomato - and keep them away from tea, which is the most common absorption blocker in Indian homes. And do not drink tea with your calcium-rich meal; the tannins bind it. Move

the 4 p.m. chai away from the curd.

### Seven days, built and counted

Each day below targets roughly 60 g of protein and 800-1,100 mg of calcium, using ordinary Indian food. Portions are for one adult woman. Roti means one medium whole-wheat or millet roti, about 30 g of flour.

| Day | Meal | What it is | Protein g | Calcium mg |
|---|---|---|---|---|
| **Day 1** | Breakfast | 2 moong dal chillas, 150 g curd, 1 tbsp til chutney | 15 | 290 |
| | Lunch | Rajma (1 katori), rice (1 cup), bhindi sabzi, salad | 16 | 160 |
| | Snack | 200 ml milk, 10 almonds | 9 | 270 |
| | Dinner | Paneer bhurji (60 g paneer), 2 ragi rotis, palak sabzi | 19 | 330 |
| | | Day total | 59 | 1050 |
| **Day 2** | Breakfast | Besan chilla (50 g besan), 200 ml milk, 1 orange | 19 | 330 |
| | Lunch | Fish curry (100 g fish), rice, salad | 25 | 180 |
| | Snack | Roasted chana (30 g), tea | 6 | 60 |
| | Dinner | Dal (1 katori), 2 rotis, lauki sabzi, 150 g curd | 18 | 300 |
| | | Day total | 68 | 870 |
| **Day 3** | Breakfast | 2-egg bhurji, 1 multigrain roti, 100 ml milk | 18 | 300 |
| | Lunch | Chole (150 g cooked), 2 rotis, cucumber salad, 100 g curd | 20 | 240 |
| | Snack | Sattu drink (30 g sattu) | 6 | 60 |
| | Dinner | Soya chunk curry (30 g dry), rice, methi sabzi | 22 | 220 |
| | | Day total | 66 | 820 |
| **Day 4** | Breakfast | Oats (40 g) with 200 ml milk and 1 tbsp chia | 14 | 320 |
| | Lunch | Chicken curry (100 g), 2 rotis, salad | 27 | 90 |
| | Snack | Paneer cubes (50 g), green tea | 9 | 105 |
| | Dinner | Moong dal khichdi, 150 g curd, bhindi sabzi | 16 | 300 |
| | | Day total | 66 | 815 |
| **Day 5** | Breakfast | 3 idli, sambar, 150 g curd | 16 | 280 |
| | Lunch | Rajma chawal, salad, 100 g curd | 20 | 200 |
| | Snack | 1 til-gud laddoo, 200 ml milk | 8 | 320 |
| | Dinner | Palak paneer (60 g paneer), 2 rotis | 18 | 330 |
| | | Day total | 62 | 1130 |
| **Day 6** | Breakfast | Poha with peanuts, 2 boiled eggs | 16 | 120 |
| | Lunch | Dal, rice, drumstick leaves sabzi, 150 g curd | 18 | 330 |
| | Snack | 150 g curd with 10 g jaggery | 5 | 230 |
| | Dinner | Grilled fish (120 g), 2 ragi rotis, salad | 28 | 220 |
| | | Day total | 67 | 900 |
| **Day 7** | Breakfast | 2 ragi dosa, coconut chutney, 2 eggs, 200 ml milk | 22 | 350 |
| | Lunch | Vegetable pulao, paneer (50 g), 150 g curd | 18 | 300 |
| | Snack | Sprouts chaat (100 g) | 6 | 60 |
| | Dinner | Dal (light makhani), 2 rotis, salad | 15 | 200 |

| Day | Meal | What it is | Protein g | Calcium mg |
|---|---|---|---|---|
| | | Day total | 61 | 910 |

Treat this as a template rather than a rulebook. Swap fish for chicken, paneer for tofu, rajma for chana. What must survive the swap is the shape: protein at all four eating occasions, a dairy or til or ragi source of calcium every day, and vegetables taking up half the plate.

## The supplement decisions, made simple

- Calcium: supplement only the gap. If your food gives you 700 mg, a 500 mg tablet covers you. Do not exceed about 1,200-1,500 mg total, and split doses above 500 mg. Calcium citrate suits people on acid-reducing medication.
- Vitamin D: 800-1,000 IU a day for maintenance is a common approach; higher corrective doses for documented deficiency, prescribed and rechecked at three to four months.
- B12: relevant if you are vegetarian or vegan, and it is a common cause of fatigue and tingling that gets blamed on menopause. Test it.
- Iron: only after a ferritin test, and only if low. Perimenopausal heavy bleeding makes this genuinely common.
- Protein powder: a convenience, not a requirement. The 2024 guidelines specifically advise getting protein from food combinations rather than supplements to build muscle.
- Omega-3: reasonable for heart health if you eat little fish; not a treatment for flushes.

## Weddings, festivals and eating out

A plan that only works on ordinary Tuesdays is not a plan in India. The next wedding, the Diwali sweets, the Sunday biryani and the office lunch are not exceptions to the year - they are the year. Five rules keep the damage contained.

- Eat protein before you go. A katori of curd or a piece of paneer at home means you arrive able to choose rather than hungry enough to eat the fried starter course.
- Take the grilled, tandoori and dry options first. Paneer tikka, chicken tikka, kebab, roasted chana and salad before the puri and the pakora. Buffets fail people at the first pass, not the third.
- One sweet, sitting down. Festival sweets are concentrated sugar and ghee; the 2024 guidelines cap added sugar at under 5% of energy, roughly 20-25 g a day, and two kaju katli can use most of it. Choose the one you actually love and skip the ones you do not.
- Watch salt, not just sugar, if your blood pressure is up. Papad, pickle, packaged namkeen, restaurant gravies and schezwan sauces carry most of the day's 5 g before the main course arrives.
- Keep the day's structure. A wedding dinner at 11 p.m. is easier on a body that had dal and curd at 1 p.m. and walked 20 minutes in the evening. Fasting all day to 'save' for the feast reliably produces overeating and a worse night's sleep.

Two practical details for the season. If you are on a transdermal patch, pack spares - a patch that peels in a warm mandap at 2 a.m. is a missed dose. And if you are standing for four hours in heels at a function, take flat shoes in the car and change. Joint pain in this decade is real, and it is not a reason to skip the function - it is a reason to plan the footwear.

# Part 14. The twelve-week strength plan

This is the part of the book with the largest long-term payoff and the smallest chance of being prescribed to you. Nothing here needs a gym, a machine, or more than 30 minutes, three times a week.

Why strength and not just walking: bone remodels in response to load, muscle grows in response to resistance, and both are what menopause takes fastest. Walking is good for your heart and mood and should stay in your week - but it will not rebuild a hip or a shoulder.

## Before you start

- Get clearance if you have known heart disease, uncontrolled blood pressure, a recent fracture, severe osteoporosis, or you have not exercised in years and are over 60.
- The equipment is your body, a chair, a wall, and two 1-litre water bottles that become 2-litre bottles, then a filled backpack.
- The rule that prevents injury and guarantees progress: add difficulty only when the last two repetitions of the last set feel hard but doable. Write it down.
- Pain rules: muscle burn is fine. Sharp joint pain, chest pain, dizziness, or pain that persists the next day means stop and get it checked.
- Never hold your breath. Exhale on the effort.

## Weeks 1-4: foundation

Two sessions a week, 25 minutes. Learn the movements before you load them. Rest 60-90 seconds between sets.

| Exercise | How | Sets x reps |
|---|---|---|
| **Sit-to-stand** | From a chair, stand without using hands, sit slowly | 3 x 8 |
| **Wall push-up** | Hands on wall at shoulder height, body straight | 3 x 8 |
| **Backpack row** | Hinge forward, pull a loaded bag to your ribs | 3 x 10 |
| **Hip hinge** | Soft knees, push hips back, flat back, return | 3 x 8 |
| **Calf raise** | Hold the wall, rise onto toes, lower slowly | 3 x 12 |
| **Glute bridge** | On your back, knees bent, lift hips, squeeze | 3 x 10 |
| **Standing balance** | One foot, hold a counter, 20 seconds each side | 2 x 20 sec |

Plus 20-30 minutes of walking on most other days. Aim for 7,000 steps as a starting target, not a law.

## Weeks 5-8: build

Three sessions a week. Same movements, more difficulty: lower the chair less, add water bottles, slow the lowering phase to three seconds.

| Exercise | Progression | Sets x reps |
|---|---|---|
| **Squat to chair** | Hold 2 x 1 L bottles at chest | 3 x 10 |
| **Incline push-up** | Hands on a table instead of the wall | 3 x 8 |
| **Row with bottles** | Two bottles, pull to ribs, squeeze shoulder blades | 3 x 10 |
| **Step-up** | Onto a 15 cm step, alternate legs | 3 x 8 each |
| **Single-leg calf raise** | One foot, fingertip support | 3 x 10 each |
| **Dead bug** | On your back, opposite arm and leg, slow | 3 x 8 each |
| **Farmer carry** | Two full bottles, walk 20 metres | 3 rounds |

Add one impact session a week if your bones and joints allow it: 10 minutes of marching in place, small hops, or stair climbing. Impact is what signals bone to rebuild, and it is the element most missing from Indian women's exercise.

### Weeks 9-12: progress

Three sessions a week, 35 minutes, plus one mobility session. Loads go up: a backpack with 4-6 kg for squats and rows, 3-second lowering on every movement, and one set taken close to failure per exercise.

| Session | Content |
|---|---|
| **A (Monday)** | Goblet squat with backpack 3 x 10, incline push-up 3 x 8, row 3 x 10, calf raise 3 x 12 |
| **B (Wednesday)** | Hip hinge with backpack 3 x 10, step-up 3 x 8 each, shoulder press with bottles 3 x 8, dead bug 3 x 8 |
| **C (Friday)** | Sit-to-stand with weight 3 x 10, reverse lunge holding chair 3 x 6 each, farmer carry 4 rounds, plank 3 x 20 sec |
| **Mobility (any day)** | 10 minutes: hip flexor stretch, chest opener in a doorway, thoracic rotation, ankle circles, pelvic floor coordination |

At the end of twelve weeks, repeat the assessment: how many sit-to-stands in 30 seconds, how long you can hold a plank, how far you can carry two full bottles, and your waist measurement. Write them next to your starting numbers in Part 17. Progress you can see is the only thing that keeps a plan alive.

### The pelvic floor, done properly

Pelvic floor exercises are prescribed constantly and taught badly. The correct action is a lift and squeeze as if you are stopping urine mid-flow and holding wind at the same time - you should feel a lift, not a bearing down. Hold for 3-5 seconds, release fully for the same time, and do 10 repetitions, three times a day. Do not practise by stopping urine mid-flow regularly; that can cause incomplete emptying. If you cannot feel a lift, ask for a physiotherapist - pelvic floor physiotherapy exists in most Indian cities now, and it works.

# Part 15. Work, family and money

India has no national law that gives menopause leave or requires workplaces to have a menopause policy as of late 2026. That is the honest starting point, and it shapes the strategy: what you are doing in this chapter is negotiating, not claiming a right. The good news is that a small number of states and employers are moving - Maharashtra built menopause clinics into its public health system, Karnataka announced a statewide programme, and several large private employers have started adding menopause to their health benefits - so the conversation is easier to start than it was five years ago.

## At work

A 2026 study of employed women in southern India found that 73% reported moderate to severe menopausal symptoms and that work ability was measurably reduced, with women in unskilled roles affected most. A separate analysis of workplace posts found confidence and self-image taking the biggest hit. In other words: this is a performance issue, not a private inconvenience, and it is legitimate to raise it as one.

Three practical moves, in order of difficulty.

- Manage the room before the meeting. A desk fan, a cotton layer you can remove, water within reach, and a seat away from the projector heat solve a surprising share of the problem without anyone being told anything.
- Change the schedule, not the job. Ask for meetings before 2 p.m. where possible, or for the option to take a 20-minute break in the afternoon. Frame it as protecting your best hours, which is true.
- Have the conversation with one person, not the whole office. HR or your manager, in writing, with the outcome you want stated plainly.

> Script for a manager: 'I am going through perimenopause, which for the next year or so means some hot flushes and disturbed sleep. It does not affect my work quality, but mornings are my best hours and late-afternoon meetings are harder. Can we move the 4 p.m. review to 11 a.m.? That is the only change I am asking for.'

If your employer has an employee assistance programme, use it - it usually includes counselling at no cost, and it is confidential. If you have group health insurance, read the policy for what outpatient consultations and diagnostics cover; many Indian policies cover hospitalisation generously and outpatient care barely at all, which is why the tests in Part 4 often come out of your own pocket.

## At home

The most common family failure is not hostility. It is invisibility. Symptoms get read as mood, and mood gets read as a character problem, and the woman absorbs the story that she has become difficult.

Two conversations change the household more than any leaflet.

> With a partner: 'For the next year or two my body is going through a hormonal transition. The main things are disturbed sleep, joint pain, and flushes. It is not about you and it is not permanent. What helps me most is that I get one uninterrupted hour to exercise three times a week, and that we do not discuss anything difficult after 10 p.m.'

> With your children, in one sentence: 'Amma is going through a stage called perimenopause - it is like puberty in reverse, it is normal, and it lasts a few years.' Naming it once removes the shame from the house for the next generation, which is the part that lasts.

If there is an elder in the house who insists it is 'heat in the body' and prescribes powders, the useful move is not an argument. It is a shared appointment. Let a doctor say the words. It costs one consultation and saves a year of conflict.

### Money

- Generic oral oestradiol and micronized progesterone are inexpensive in India; transdermal patches and gels cost more and are less consistently stocked. Ask about alternatives before you decide on cost grounds alone.
- Diagnostics are the bigger expense: DXA, mammogram, lipid and HbA1c panels. Public sector facilities and district hospitals often provide these at a fraction of private rates, and the NP-NCD programme provides breast and cervical screening free at Health and Wellness Centres.
- Do not fund it by cutting protein. The cheapest interventions in this book - dal, curd, til, ragi, walking, resistance training - are also the ones with the best evidence.
- Keep a single folder, physical or digital, with every report, prescription and date. It saves repeated testing, which is where money actually leaks.

# Part 16. Your screening calendar, 40 to 65

This is the page to photograph and keep. It combines what India's public health programme offers free, what private practice adds, and when each thing is due. Intervals are typical; your doctor may adjust them for family history or findings.

| What | When | Where in India | Notes |
|---|---|---|---|
| **Blood pressure** | Every visit; monthly at home after 40 | Any pharmacy or clinic | Home readings beat single clinic readings |
| **Weight and waist** | Monthly | At home | Waist above 80 cm flags metabolic risk in Indian women |
| **HbA1c or fasting glucose** | Every 1-3 years; annually if borderline | Any lab; free at HWCs under NP-NCD | The single best sugar screen |
| **Lipid profile** | Every 1-2 years after 40 | Any lab | LDL rises through the transition |
| **Haemoglobin and ferritin** | Now, and if bleeding is heavy | Any lab | Iron deficiency is common and curable |
| **TSH** | Once, then as indicated | Any lab | Mimics every menopausal symptom |
| **Vitamin D and calcium** | Once, then after 3-4 months of treatment | Any lab | Deficiency is widespread |
| **DXA bone density** | At menopause, or now if never done | District hospitals, private centres | One baseline scan guides a decade |
| **Clinical breast examination** | Every 5 years, ages 30-65 (national programme); annually in private practice | Health and Wellness Centre, free | The NP-NCD protocol is CBE once every five years |
| **Mammogram** | From 40, every 1-2 years; earlier with family history | Private centres, some district hospitals | Discuss density and family history with your doctor |
| **Cervical screening** | Every 3-5 years, ages 30-65 | HWC (VIA, free) or lab (Pap/HPV) | VIA is less accurate after menopause - ask for Pap or HPV testing |
| **Dental check** | Every 6 months | Any dentist | Gum disease accelerates with dry mouth and bone loss |
| **Eye check** | Every 2 years after 40 | Any optometrist | Vision affects fall risk directly |
| **Mood screen** | At every review | Any doctor | Two questions are enough to start |

Two India-specific notes that change what you should ask for. First, the national programme's cervical screening uses visual inspection with acetic acid, which is effective in younger women but less reliable after menopause because the transformation zone recedes into the canal. If you are over 50, ask for a Pap smear or an HPV test instead, even if you pay for it. Second, the programme's breast screening is clinical examination every five years for women aged 30-65; a mammogram is not part of the basic package, so if you are over 40 and can afford one, that is a private decision worth making.

## How to read your own report

Lab reference ranges differ slightly between laboratories, so always read the range printed on your own report. The bands below are the ones used in Indian clinical practice and are enough to tell you whether to act.

| Test | Normal | Act on it |
|---|---|---|
| **Blood pressure** | Below 120/80 | 130s and above: home log and a plan. Indian guidelines diagnose hypertension at 140/90; recent international guidance moves the threshold to 130/80 |
| **Fasting glucose** | Below 100 mg/dL | 100-125 prediabetes; 126 and above needs a diabetes diagnosis confirmed on repeat |
| **HbA1c** | Below 5.7% | 5.7-6.4% prediabetes; 6.5% and above diabetes |
| **LDL cholesterol** | Below 100 mg/dL | Below 70 if you already have heart disease or diabetes; the number that responds to diet and statins |
| **Triglycerides** | Below 150 mg/dL | Rises with refined carbohydrate, alcohol and visceral fat |
| **HDL cholesterol** | Above 50 mg/dL in women | Low HDL tracks with inactivity and smoking |
| **Haemoglobin** | 12 g/dL and above | Below 12 needs investigation; below 10 needs treatment now |
| **Ferritin** | Above 30 ng/mL | Below 30 suggests iron deficiency even when haemoglobin is normal |
| **TSH** | 0.4-4.0 mIU/L typically | Outside range: thyroid disease, which mimics every menopausal symptom |
| **Vitamin D** | 30-100 ng/mL | 20-30 insufficient; below 20 deficient, needs correction and a recheck |
| **DXA T-score** | Above -1.0 | -1.0 to -2.5 is low bone mass; -2.5 or below is osteoporosis |
| **Waist** | Below 80 cm | 80-88 cm increased metabolic risk in Indian women; above 88 cm substantially increased |

Two habits make these numbers useful. Test at the same laboratory where possible, so the method does not change between readings. And write each result into the log in Part 17 with the date, so the trend - not the single value - drives the decision.

# Part 17. Your paperwork: trackers and the one-page sheet

Print these pages or copy them into a notebook. Four weeks of your own data is worth more in a consultation than any description you can give from memory.

## Symptom tracker, four weeks

Score each symptom 0 (none) to 4 (very severe) for the week. Add the row for a weekly total. Take the completed page to your appointment.

| Symptom | Week 1 | Week 2 | Week 3 | Week 4 |
|---|---|---|---|---|
| **Hot flushes** | | | | |
| **Night sweats** | | | | |
| **Sleep disturbance** | | | | |
| **Joint or muscle pain** | | | | |
| **Exhaustion** | | | | |
| **Low mood** | | | | |
| **Anxiety or palpitations** | | | | |
| **Irritability** | | | | |
| **Memory or concentration** | | | | |
| **Vaginal dryness or pain** | | | | |
| **Urinary symptoms** | | | | |
| **Headache** | | | | |
| **Weekly total** | | | | |

## Numbers log

| Date | BP | Weight | Waist cm | HbA1c | LDL | Vit D | Notes |
|---|---|---|---|---|---|---|---|
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## Strength log

| Week | Sessions done | Squat load | Row load | Sit-to-stands in 30 sec | Notes |
|---|---|---|---|---|---|
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### The one-page sheet for your appointment

Fill this in before you go. Hand it over if the consultation is short. It works because it answers the questions a doctor would ask, in the order they ask them.

- Age, and date of last period (or 'irregular since ____').
- My three worst symptoms, with how many days a week they occur.
- What I have already tried, and what happened.
- My current medicines, supplements and doses.
- Family history: breast, ovarian, colon cancer; heart attack or stroke before 60; hip fracture; diabetes; thyroid.
- My last results: BP, HbA1c, lipids, haemoglobin, TSH, vitamin D, DXA if done, last mammogram and Pap with dates.
- What I want from today: diagnosis, tests, treatment options, or a referral. Say it out loud at the start.

> The five questions to ask before you leave the room: What do you think is causing this? What test will change the decision? What are the options, including doing nothing? What should I expect in three months? When do I come back, and what would make it urgent?

# Part 18. When to stop reading and get seen today

Everything in this book is about a transition that is uncomfortable but not dangerous. The list below is different. These are the situations where the right response is a same-day appointment, an emergency department, or a phone call - not a tracker, not a supplement, not next month.

### Go now, the same day

- Any vaginal bleeding after twelve consecutive months without a period.
- Bleeding after sex, or bleeding between periods that is new.
- Bleeding that soaks a pad an hour for two hours or more, or clots larger than a rupee coin.
- Chest pain, pressure or tightness, especially with sweating, nausea or breathlessness - in women the presentation is often atypical.
- Sudden breathlessness, or a swollen, painful, hot calf - possible clot, particularly on oral oestrogen.
- Sudden severe headache, or a new weakness, facial droop, or difficulty speaking.
- A new breast lump, skin dimpling, nipple discharge or a nipple that has turned in.
- Fainting, or a racing heart with dizziness.
- Thoughts of harming yourself, or that life is not worth living.

### Book an appointment this week

- A joint that is hot, swollen and locked, or morning stiffness lasting over an hour.
- Three or more urinary infections in a year.
- Unintentional weight loss, or a new persistent cough.
- A fracture from a fall from standing height - that is a fragility fracture whatever the scan says.
- Symptoms that have not improved after three months of treatment.
- New or worsening low mood lasting more than two weeks.
- Any bleeding on hormone therapy after the first six months.

### The four myths to drop

| Myth | What is actually true |
|---|---|
| **'Hormones cause cancer, so never take them'** | Risk depends on the regimen, the route, the duration and when you start. For most women under 60 with symptoms, the balance favours treatment. The FDA moved away from blanket warnings in 2025-26 for exactly this reason |
| **'Menopause is over once your periods stop'** | Flushes can persist for years, and vaginal and bone changes progress indefinitely without treatment |
| **'Just take calcium and you will be fine'** | Calcium alone does not rebuild bone. Load, protein, vitamin D and, where indicated, drug treatment are what change fracture risk |
| **'It is all in your head'** | Oestrogen receptors are in the brain, the bone, the vessel wall and the bladder. The symptoms are physiological, and the mood symptoms are among the most common in Indian data |

# Part 19. Questions women actually ask

## About timing

**My mother had no symptoms. Why do I?** Symptom experience varies enormously between women and between populations. Indian studies show a wide range - hot flushes reported anywhere from 28% to 69% depending on the sample. Genetics, BMI, smoking, sleep and stress all shift it. Your mother's experience is not a prediction.

**I am 41. Is it too early?** No. Perimenopause commonly begins in the early forties, and the Indian average start is around 44.7 years. Under 40, stopping periods needs investigation for premature ovarian insufficiency rather than reassurance.

**My periods are still regular. Can I still be perimenopausal?** Yes. Symptoms often precede cycle change by months. Regular cycles do not exclude the transition.

**How long does this last?** The transition averages about four years, but symptom duration varies widely, and a minority of women have flushes for a decade or more. Vaginal and bone changes do not resolve on their own.

## About tests

**Should I get my hormones tested?** Usually not, for diagnosis. FSH fluctuates too much to be reliable in your forties. It is useful in suspected premature ovarian insufficiency, after hysterectomy, and when cycles are masked by hormonal contraception.

**My FSH is normal. Does that mean nothing is wrong?** No. It means that sample was normal. Diagnosis in this decade is clinical - symptoms plus cycle pattern.

**What should I insist on?** Blood pressure, HbA1c, lipids, haemoglobin with ferritin, TSH, vitamin D, and a DXA scan if you have not had one. Those are the tests that change outcomes.

## About hormone therapy

**Will I gain weight on it?** Trials do not show that hormone therapy causes weight gain. What happens without it - muscle loss and visceral fat gain - is a bigger driver. Some women notice bloating early, which usually settles.

**Do I have to stop at five years?** No. There is no mandatory stopping age or duration. Continuation is reviewed annually, weighing symptoms, bone benefit and personal risk.

**Can I use it if I have had breast cancer?** Systemic hormone therapy is generally contraindicated after breast cancer. Local vaginal oestrogen is often still possible after discussion with your oncology team, and non-hormonal options remain.

**What about 'bioidentical' hormones?** The word has no regulatory meaning in India. Compounded preparations are not standardised for dose or purity. Approved estradiol and micronized progesterone are what the guidelines refer to when they discuss hormone therapy.

**Will it protect my heart?** No. It neither protects nor harms the heart when started in the window. Take it for symptoms and bone, and manage cardiovascular risk separately.

## About daily life

**I have stopped sleeping. Is a sleeping pill the answer?** Rarely as a first move. Treat night sweats if they are waking you, and run the behavioural reset in Part 9 for four weeks. Sleeping pills are for short-term crisis, not for a two-year transition.

**Is it normal to feel like a different person?** Mood, sleep, body and libido all shifting at once is disorienting. It is also common - depressive mood was reported by 70% in one Indian series. That is a reason to get help, not a reason to feel ashamed.

**Do I still need contraception?** Yes, until twelve consecutive months without a period. The WHO guidance is explicit on this, and pregnancy in perimenopause is uncommon but real.

**Will exercise really help, or is that just something doctors say?** It helps specifically and measurably: it preserves muscle and bone, improves sleep and mood, and reduces metabolic risk. It does not eliminate flushes, and it is not a substitute for treatment if symptoms are severe. It is the base layer, not the whole building.

**What about yoga and pranayama?** Modest but real benefits for sleep, stress and how much symptoms bother you, with small effects on flush frequency. Worth doing alongside treatment, not instead of it.

**Does Ayurveda help?** The classical texts describe menopause as Rajonivritti, a normal transition around fifty, and the 2026 guidelines note this as a legitimate cultural framework rather than dismissing it. Some preparations may help sleep and general wellbeing. Two cautions: several herbal products are unstandardised for dose and purity, and some interact with thyroid, diabetes and blood-thinning medicines. Tell every doctor everything you take, including churnas and kadhas.

**Should I tell my employer?** You are not obliged to. But if symptoms are affecting your best working hours, telling one trusted person - HR or a manager - usually costs less than the performance dip that goes unexplained. Part 15 has the wording.

**Will symptoms come back if I stop treatment?** Flushes usually return when systemic hormone therapy stops, because the underlying transition has not changed; they often settle over months. Vaginal symptoms return fairly quickly if local oestrogen stops, which is why it is used long term rather than as a course.

**What if I cannot afford all these tests?** Prioritise in this order: blood pressure, haemoglobin with ferritin, HbA1c, and a DXA scan. The first three are cheap or free at public facilities; the DXA is the one worth saving for. Skip the hormone panels, the hair-mineral analyses and the food-intolerance tests - they are expensive and rarely change anything.

**I feel alone in this. Is that normal?** It is the most common thing women say, and it is a consequence of silence rather than of your situation. Naming it to one other woman - a friend, a sister, a colleague - reliably changes how heavy it feels.

# Part 20. Where this comes from

Every factual claim in this book is drawn from the sources below. Where Indian and international guidance differ, both positions are stated rather than blended. Nothing here is invented, and no figure has been rounded into a different claim.

### Guidelines and official bodies


- Indian Menopause Society. Clinical Practice Guidelines for Menopause 2026 - Executive Summary and Recommendations. Journal of Mid-life Health, February 2026, Volume 17, Supplement 1, pages S12-S116. Includes the 'Start at 35' screening approach, the grading of cardiometabolic acceleration around the final menstrual period, transdermal oestrogen preference, local oestrogen doses, and premature ovarian insufficiency management.
- Indian Menopause Society. Clinical Practice Guidelines on Menopause 2019-2020, and Clinical Practice Guidelines on Postmenopausal Osteoporosis 2019-2020 - Executive Summary and Recommendations.
- World Health Organization. Menopause fact sheet, 16 October 2024.
- US Food and Drug Administration. Request for labeling changes for menopausal hormone therapies, 10 November 2025; and the subsequent approval of updated labeling, February 2026.
- US Food and Drug Administration. Drug Trials Snapshot: Lynkuet (elinzanetant), approval date 24 October 2025.
- ICMR-National Institute of Nutrition. Dietary Guidelines for Indians, 2024, and Recommended Dietary Allowances and Estimated Average Requirements, 2020.
- Ministry of Health and Family Welfare, Government of India. National Programme for Prevention and Control of Non-Communicable Diseases, operational guidelines 2023-2030, and NP-NCD guidance on cancer screening at Health and Wellness Centres.


### Indian epidemiology and clinical studies


- Indian Menopause Society pan-India survey on age at menopause and its determinants, published in the Journal of Mid-life Health - mean age at natural menopause 46.2 years.
- Multicentric study of hot flushes in 1,479 Indian women aged 40-60, Journal of Mid-life Health, 2025 - prevalence 37.6%, with 2.2% severe.
- Modified Menopause Rating Scale study of 300 postmenopausal women, Lucknow - joint and muscular discomfort 87%, depressive mood 70%, hot flushes 53%.
- Cross-sectional studies of postmenopausal women in rural Delhi and South Canara, and a rural central India cohort, on symptom prevalence and average symptom counts.
- Systematic review of menopausal symptoms among Indian women, 2024, covering 23 studies and 5,074 participants.
- Study of bone health in rural South Indian postmenopausal women - mean dietary calcium around 420 mg a day, vitamin D deficiency in more than half, osteoporosis at the femoral neck in 22% and at the lumbar spine in 39%.
- Cross-sectional study of employed women in southern India, 2026, on menopausal symptoms and work ability.


### Context and policy


- The Lancet menopause series, 2024, including Hickey and colleagues on menopause as part of healthy ageing.
- Kantar, India in Search 2026 - reported search growth for perimenopause and for strength training in older adults.
- Reporting on Karnataka's 'Ruthuthare' programme and its planned women's health policy, and on Maharashtra's menopause clinics and the number of women screened, 2026.

## A closing note

You are not being difficult, and you are not falling apart. You are in a decade that Indian medicine has only recently started to write guidelines for, in a country where the average woman reaches this transition earlier than the textbooks of the last generation assumed.

The plan is not complicated. Get the numbers measured. Treat what is treatable, including the symptoms you were told to endure. Put protein, calcium, til, ragi and resistance training into an ordinary week. Say the word out loud at home and at work. And review it once a year instead of once a crisis.

That is the whole book, really. The rest is detail.

## Disclaimer

This book is educational and does not replace individual medical advice. Menopause, hormone therapy, bone health and cardiovascular risk all depend on your personal history, examination and test results. Doses, products and intervals mentioned here are the ones discussed in published guidelines, not a prescription for you. Consult a qualified doctor before starting, stopping or changing any treatment, and seek urgent care for any of the symptoms listed in Part 18.